ondansetron (Rx)

Brand and Other Names: Zofran (DSC), Zofran ODT (DSC), Zuplenz (DSC)
Classes: Antiemetics, Selective 5-HT3 Antagonists

Dosing & Uses

ADULT

Dosage Forms & Strengths

injectable solution

- 2mg/mL

tablet

- 4mg
- 8mg
- 24mg

oral solution

- 4mg/5mL

orally disintegrating tablets

- 4mg
- 8mg
- 16mg
- 24mg

Chemotherapy-Induced Nausea & Vomiting

Prophylaxis

PO

- Moderately emetogenic chemotherapy: 8 mg started 30 minutes before chemotherapy, then q12hr for 1-2 days after chemotherapy
- Highly emetogenic chemotherapy: 24 mg started 30 minutes before chemotherapy
- Zofran ODT: Using dry hands, carefully remove from blister pack immediately before use

IV

- 0.15 mg/kg over 15 min administered 30 min before chemotherapy, then 4 and 8 hr after first dose; not to exceed 16 mg (32 mg no longer recommended because of increased risk of QT prolongation)

Postoperative Nausea & Vomiting

Prophylaxis

4 mg IV/IM immediately before anesthesia or after procedure or 16 mg PO 1 hr before anesthesia; patients >80 kg may need additional 4 mg IV

Radiation-Induced Nausea & Vomiting

Prophylaxis

Total body radiation therapy: 8 mg PO 1-2 hours before radiation therapy; administered each day

Single high-dose fraction therapy to abdomen: 8 mg PO 1-2 hr before radiation therapy; administer subsequent doses every 8 hr after first dose 1-2 days after completion of therapy

Daily fractions to abdomen: Administer 8 mg PO 1-2 hr before radiotherapy; administer subsequent doses every 8 hr after first dose each day radiotherapy is given

Dosage Modifications

Renal impairment: Dose adjustment not necessary

Severe hepatic impairment (Child-Pugh score ≥10): Not to exceed 8 mg/day

Cholestatic Pruritus (Off-label)

8 mg divided q12hr or 8 mg q8-12hr PO for 7 days up to 5 months

Alternatively, 4-8 mg intermittent short-term IV dosing used in adults; single dose of 4 mg single dose used in pregnancy

Uremic Pruritus (Off-label)

8 mg divided q12hr or 8 mg q8-12hr PO for 14 days up to 5 months

Spinal Opioid-Induced Pruritus (Off-label)

Prophylaxis: 4-8 mg IV 20-30 min prior to spinal opioid therapy; may repeat dosing at 12, 24, 36, 48 hr after spinal opioid dosing

Treatment: 4-8 mg IV

Rosacea (Off-label)

4-8 mg PO q12hr for up to 3 weeks

Alternatively, 12 mg IV daily for 4 days

Hyperemesis Gravidarum

10 mg IV q8hr PRN

PEDIATRIC

Dosage Forms & Strengths

injectable solution

- 2mg/mL

tablet

- 4mg
- 8mg
- 24mg

oral solution

- 4mg/5mL

orally disintegrating tablets

- 4mg
- 8mg
- 16mg
- 24mg

Chemotherapy-Induced Nausea & Vomiting

Prophylaxis

PO

- <4 years old: Safety and efficacy not established
- 4-12 years: 4 mg started 30 min before chemotherapy, then 4 and 8 hr after first dose, then q8hr for 1-2 days after chemotherapy
- >12 years: 8 mg started 30 min before chemotherapy, then q12hr for 1-2 days after chemotherapy, or single dose of 24 mg

IV

- <6 months: Safety and efficacy not established
- ≥6 months: 0.15 mg/kg over 15 min administered 30 min before chemotherapy, then repeated 4 and 8 hr after first dose; not to exceed 16 mg/dose (32 mg no longer recommended because of increased risk of QT prolongation)

Postoperative Nausea & Vomiting

Prophylaxis

1 month-12 years

- <40 kg, 0.1 mg/kg IV
- >40 kg, 4 mg IV

>12 years

- 4 mg IV/IM immediately before anesthesia or after procedure or 16 mg PO 1 hr before anesthesia; patients >80 kg may need additional 4 mg IV

Acute Gastroenteritis (Off-Label)

Routine use is not recommended (CDC)

May consider a single, one-time oral dose for severe gastroenteritis

8-15 kg: 2 mg PO once

>15 to 30 kg: 4 mg PO once

>30 kg: 8 mg PO once

Reference: N Engl J Med. 2006;354(15):1698-1705

Chemotherapy-Induced Nausea & Vomiting (Orphan)

Ondansetron inhalation powder: Prevention of chemotherapy-induced nausea and vomiting due to highly emetogenic chemotherapy in pediatric patients (aged birth through 16 yr)

Sponsor

- Luxena Pharmaceuticals, Inc; 1400 Coleman Avenue, Suite D27; Santa Clara, California 95050

Interactions

Interaction Checker

Enter a drug name to check for any interactions. Type a drug, OTC, or herbal and ondansetron
No Interactions Found
Interactions Found

Contraindicated

Serious

Significant - Monitor Closely

Minor

All Interactions Sort By: Severity

Contraindicated (4)

- apomorphine
  apomorphine, ondansetron. Mechanism: unspecified interaction mechanism. Contraindicated. Profound hypotension and loss of consciousness reported when 5HT3 antagonists are coadministered with apomorphine. Additionally, ondansetron may cause additive QT prolongation with apomorphine.

- dronedarone
  dronedarone will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Contraindicated.

  dronedarone and ondansetron both increase QTc interval. Contraindicated. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- lefamulin
  lefamulin will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Contraindicated. Lefamulin is contraindicated with CYP3A substrates know to prolong the QT interval.

- posaconazole
  posaconazole will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Contraindicated.

Serious (164)

- adagrasib
  adagrasib, ondansetron. Either increases effects of the other by QTc interval. Avoid or Use Alternate Drug. Each drug prolongs the QTc interval, which may increased the risk of Torsade de pointes, other serious arryhthmias, and sudden death. If coadministration unavoidable, more frequent monitoring is recommended for such patients.

- alfuzosin
  alfuzosin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- almotriptan
  ondansetron, almotriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- amiodarone
  amiodarone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- amisulpride
  amisulpride and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. ECG monitoring is recommended if coadministered.

- amitriptyline
  amitriptyline and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, amitriptyline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- amoxapine
  amoxapine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, amoxapine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- anagrelide
  anagrelide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- apalutamide

apalutamide will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug. Coadministration of apalutamide, a strong CYP3A4 inducer, with drugs that are CYP3A4 substrates can result in lower exposure to these medications. Avoid or substitute another drug for these medications when possible. Evaluate for loss of therapeutic effect if medication must be coadministered. Adjust dose according to prescribing information if needed.

- aripiprazole
  aripiprazole and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- arsenic trioxide
  arsenic trioxide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- artemether
  artemether and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- artemether/lumefantrine
  artemether/lumefantrine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- asenapine
  asenapine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- asenapine transdermal
  asenapine transdermal and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- atomoxetine
  atomoxetine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- azithromycin
  azithromycin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- buprenorphine
  buprenorphine and ondansetron both increase QTc interval. Avoid or Use Alternate

Drug.

- buprenorphine buccal
  buprenorphine buccal and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- buprenorphine subdermal implant
  buprenorphine subdermal implant and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- buprenorphine transdermal
  buprenorphine transdermal and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- buprenorphine, long-acting injection
  buprenorphine, long-acting injection and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- ceritinib
  ceritinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

  ceritinib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- chlorpromazine
  chlorpromazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- ciprofloxacin
  ciprofloxacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- citalopram
  citalopram and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, citalopram. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- clarithromycin
  clarithromycin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Potential for increased ondansetron levels.

clarithromycin will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- clomipramine
  clomipramine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, clomipramine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- clozapine
  clozapine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- crizotinib
  crizotinib and ondansetron both decrease QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- dasatinib
  dasatinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- degarelix
  degarelix and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- desflurane
  desflurane and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- desipramine
  desipramine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, desipramine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- desvenlafaxine
  ondansetron, desvenlafaxine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- disopyramide
  disopyramide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- dofetilide
  dofetilide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- dolasetron
  dolasetron and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- donepezil
  donepezil and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- dordaviprone
  dordaviprone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Additive QTc interval prolongation may increase the risk for ventricular tachyarrhythmias (eg, Torsades de pointes) or sudden death. If coadministration cannot be avoided, separate the administration time of these agents and increase the frequency of ECG and electrolyte monitoring.

- doxepin
  ondansetron, doxepin. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- droperidol
  droperidol and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- duloxetine
  ondansetron, duloxetine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- efavirenz
  efavirenz and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- eletriptan
  ondansetron, eletriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- eliglustat
  eliglustat and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- encorafenib
  encorafenib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- entrectinib
  entrectinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- enzalutamide
  enzalutamide will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- erdafitinib
  erdafitinib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If coadministration unavoidable, separate administration by at least 6 hr before or after administration of P-gp substrates with narrow therapeutic index.

- eribulin
  eribulin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- erythromycin base
  erythromycin base and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- erythromycin ethylsuccinate
  erythromycin ethylsuccinate and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- erythromycin lactobionate
  erythromycin lactobionate and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- erythromycin stearate
  erythromycin stearate and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- escitalopram
  escitalopram and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

ondansetron, escitalopram. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- ezogabine
  ezogabine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- fexinidazole
  fexinidazole and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid coadministration of fexinidazole with drugs known to block potassium channels or prolong QT interval.

- fingolimod
  fingolimod and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- flecainide
  flecainide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- fluconazole
  fluconazole and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Combination may increase ondansetron levels.

- fluoxetine
  fluoxetine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, fluoxetine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- fluphenazine
  fluphenazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- fluvoxamine
  fluvoxamine and ondansetron both increase serotonin levels. Avoid or Use Alternate Drug.

- foscarnet
  foscarnet and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with

concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- fosfomycin
  fosfomycin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. The risk of QT prolongation may increase in patients with electrolyte abnormalities (such as hypokalemia and hypocalcemia), patients with conditions that have potential to cause electrolyte imbalance (such as renal impairment), or patients taking other drugs affecting electrolyte balance or QT prolongation; avoid this drug in patients with known QT prolongation or ventricular arrhythmias, including a history of torsade de pointes; monitor electrolytes during treatment

- frovatriptan
  ondansetron, frovatriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- gemifloxacin
  gemifloxacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- gepotidacin
  gepotidacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. If coadministration unavoidable, monitor and correct serum electrolyte abnormalities and obtain ECG before administration and during treatment, as clinically indicated

- gilteritinib
  gilteritinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- givinostat
  ondansetron and givinostat both increase QTc interval. Avoid or Use Alternate Drug. If unable to avoid coadministration, obtain ECGs when initiating, during concomitant use, and as clinically indicated. Withhold if QTc interval >500 ms or a change from baseline >60 ms.

- glasdegib
  ondansetron and glasdegib both increase QTc interval. Avoid or Use Alternate Drug. If coadministration unavoidable, monitor for increased risk of QTc interval prolongation.

- granisetron
  granisetron and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- haloperidol
  haloperidol and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- hydroxychloroquine sulfate
  hydroxychloroquine sulfate and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- ibutilide
  ibutilide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- idelalisib
  idelalisib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug. Idelalisib is a strong CYP3A inhibitor; avoid coadministration with sensitive CYP3A substrates

- iloperidone
  ondansetron and iloperidone both increase QTc interval. Avoid or Use Alternate Drug.

- imipramine
  ondansetron, imipramine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- indacaterol, inhaled
  indacaterol, inhaled, ondansetron. QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- indapamide
  indapamide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- inotuzumab
  inotuzumab and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. If unable to avoid concomitant use, obtain ECGs and electrolytes before and after initiation of any drug known to prolong QTc, and periodically monitor as clinically indicated during treatment.

- isocarboxazid
  ondansetron, isocarboxazid. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- isoflurane
  isoflurane and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- isradipine
  isradipine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with

concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- itraconazole
  itraconazole and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- ivosidenib
  ivosidenib will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug. Avoid coadministration of sensitive CYP3A4 substrates with ivosidenib or replace with alternative therapies. If coadministration is unavoidable, monitor patients for loss of therapeutic effect of these drugs.

- lapatinib
  lapatinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Potential for increased ondansetron levels.

- levofloxacin
  levofloxacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- levomilnacipran
  ondansetron, levomilnacipran. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- lithium
  lithium and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- lopinavir
  lopinavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- lumefantrine
  lumefantrine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- macimorelin
  macimorelin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Macimorelin causes an increase of ~11 msec in the corrected QT interval. Avoid coadministration with drugs that prolong QT interval, which could increase risk for developing torsade de pointes-type ventricular tachycardia. Allow sufficient washout time of drugs that are known to prolong the QT interval before administering macimorelin.

- mefloquine

mefloquine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- methadone
  methadone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- mifepristone
  mifepristone will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- milnacipran
  ondansetron, milnacipran. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- milsaperidone
  ondansetron and milsaperidone both increase QTc interval. Avoid or Use Alternate Drug.

- moxifloxacin
  moxifloxacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- naratriptan
  ondansetron, naratriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- nilotinib
  nilotinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Potential for increased ondansetron levels.

- nortriptyline
  nortriptyline and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, nortriptyline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- octreotide
  octreotide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with

concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- ofloxacin
  ofloxacin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- olanzapine
  olanzapine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- oxaliplatin
  oxaliplatin and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- paliperidone
  paliperidone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- paroxetine
  ondansetron, paroxetine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- pazopanib
  pazopanib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- pentamidine
  pentamidine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- perphenazine
  perphenazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- phenelzine
  ondansetron, phenelzine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- pimozide

pimozide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- pitolisant
  ondansetron and pitolisant both increase QTc interval. Avoid or Use Alternate Drug.

- posaconazole
  posaconazole and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Potential for increased ondansetron levels.

- primaquine
  primaquine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- procainamide
  procainamide and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- propafenone
  propafenone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- protriptyline
  protriptyline and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

  ondansetron, protriptyline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- quetiapine
  quetiapine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- quinidine
  quinidine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- quinine

quinine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- ranolazine
  ranolazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- rasagiline
  ondansetron, rasagiline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- revumenib
  revumenib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. ECG monitoring recommended if concomitant use can?t be avoided and withhold therapy if QTc interval is >480 msec; restart therapy after QTc interval returns to <480 msec

- ribociclib
  ribociclib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- risperidone
  ondansetron and risperidone both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- rizatriptan
  ondansetron, rizatriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- romidepsin
  ondansetron and romidepsin both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- selegiline
  ondansetron, selegiline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- selegiline transdermal
  ondansetron, selegiline transdermal. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- sertraline
  ondansetron and sertraline both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

ondansetron, sertraline. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- sevoflurane
  sevoflurane and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- siponimod
  siponimod and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- solifenacin
  ondansetron and solifenacin both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- sotalol
  ondansetron and sotalol both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- sotorasib
  sotorasib will decrease the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If use is unavoidable, refer to the prescribing information of the P-gp substrate for dosage modifications.

- sumatriptan
  ondansetron, sumatriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- sumatriptan intranasal
  ondansetron, sumatriptan intranasal. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- sunitinib
  ondansetron and sunitinib both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- tacrolimus
  ondansetron and tacrolimus both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- taletrectinib
  taletrectinib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Concomitant use may result in additive QTc interval prolongation, which can

increase the risk for ventricular tachyarrhythmias (eg, Torsades de pointes) or sudden death.

- tedizolid
  tedizolid, ondansetron. Either increases effects of the other by Mechanism: pharmacodynamic synergism. Avoid or Use Alternate Drug. both increase serotonin levels; increased risk of serotonin syndrome.

- telavancin
  ondansetron and telavancin both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- tepotinib
  tepotinib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If concomitant use unavoidable, reduce the P-gp substrate dosage if recommended in its approved product labeling.

- tetrabenazine
  tetrabenazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug.

- thioridazine
  thioridazine and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- thiothixene
  ondansetron and thiothixene both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- toremifene
  ondansetron and toremifene both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- tranylcypromine
  ondansetron, tranylcypromine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- trimipramine
  ondansetron and trimipramine both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

ondansetron, trimipramine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- tucatinib
  tucatinib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug. Avoid concomitant use of tucatinib with CYP3A substrates, where minimal concentration changes may lead to serious or life-threatening toxicities. If unavoidable, reduce CYP3A substrate dose according to product labeling.

- umeclidinium bromide/vilanterol inhaled
  ondansetron increases toxicity of umeclidinium bromide/vilanterol inhaled by QTc interval. Avoid or Use Alternate Drug. Exercise extreme caution when vilanterol coadministered with drugs that prolong QTc interval; adrenergic agonist effects on the cardiovascular system may be potentiated.

- vandetanib
  vandetanib and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- vardenafil
  ondansetron and vardenafil both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- vemurafenib
  ondansetron and vemurafenib both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Alterations in ondansetron concentrations may occur with concomitant use.

- venlafaxine
  ondansetron, venlafaxine. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

- vilanterol/fluticasone furoate inhaled
  ondansetron increases toxicity of vilanterol/fluticasone furoate inhaled by QTc interval. Avoid or Use Alternate Drug. Exercise extreme caution when vilanterol coadministered with drugs that prolong QTc interval; adrenergic agonist effects on the cardiovascular system may be potentiated.

- voriconazole
  ondansetron and voriconazole both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. Potential for increased ondansetron levels.

voriconazole will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Avoid or Use Alternate Drug.

- vorinostat
  ondansetron and vorinostat both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- ziftomenib
  ondansetron and ziftomenib both increase QTc interval. Avoid or Use Alternate Drug. Avoid coadministration of ziftomenib with other drugs known to prolong the QTc interval. If unavoidable, obtain ECGs when initiating, during concomitant use, and as clinically indicated. Interrupt ziftomenib with QTc interval > 500 ms or a change from baseline is > 60 ms

- ziprasidone
  ziprasidone and ondansetron both increase QTc interval. Avoid or Use Alternate Drug. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- zolmitriptan
  ondansetron, zolmitriptan. Either increases toxicity of the other by serotonin levels. Avoid or Use Alternate Drug.

Monitor Closely (101)

- abemaciclib
  abemaciclib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- abrocitinib
  abrocitinib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- adagrasib
  adagrasib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- aficamten
  aficamten will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- albuterol
  ondansetron and albuterol both increase QTc interval. Use Caution/Monitor. Combination may increase risk of QT prolongation; monitor electrolytes and ECG changes

- albuterol inhaled
  ondansetron and albuterol inhaled both increase QTc interval. Use Caution/Monitor. Combination may increase risk of QT prolongation; monitor electrolytes and ECG

changes

- alfuzosin
  ondansetron and alfuzosin both increase QTc interval. Use Caution/Monitor.

- apixaban
  apixaban will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- arformoterol
  ondansetron and arformoterol both increase QTc interval. Use Caution/Monitor. Combination may increase risk of QT prolongation; monitor electrolytes and ECG changes

- atazanavir
  atazanavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended.

- azithromycin
  azithromycin will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- bedaquiline
  ondansetron and bedaquiline both increase QTc interval. Modify Therapy/Monitor Closely. ECG should be monitored closely

- belumosudil
  belumosudil will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- berotralstat
  berotralstat will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Monitor or titrate P-gp substrate dose if coadministered.

- bosutinib
  bosutinib increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- cannabidiol
  cannabidiol increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- capmatinib
  capmatinib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- carbamazepine
  carbamazepine will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Use Caution/Monitor.

carbamazepine will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- carfilzomib
  carfilzomib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- cenobamate
  cenobamate will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Increase dose of CYP3A4 substrate, as needed, when coadministered with cenobamate.

- chloroquine
  chloroquine increases toxicity of ondansetron by QTc interval. Use Caution/Monitor.

- cobicistat
  cobicistat will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

  cobicistat will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- crizotinib
  crizotinib increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- crofelemer
  crofelemer increases levels of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Crofelemer has the potential to inhibit CYP3A4 at concentrations expected in the gut; unlikely to inhibit systemically because minimally absorbed.

- dabrafenib
  dabrafenib will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely.

- danicopan
  danicopan will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Danicopan increases plasma concentrations of P-gp substrates; consider dose reduction of P-gp substrates where minimal concentration changes may lead to serious adverse reactions.

- daridorexant
  daridorexant will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- darunavir
  darunavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- deferasirox
  deferasirox will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- deutetrabenazine
  deutetrabenazine and ondansetron both increase QTc interval. Use Caution/Monitor. At the maximum recommended dose, deutetrabenazine does not prolong QT interval to a clinically relevant extent. Certain circumstances may increase risk of torsade de pointes and/or sudden death in association with drugs that prolong the QTc interval (eg, bradycardia, hypokalemia or hypomagnesemia, coadministration with other drugs that prolong QTc interval, presence of congenital QT prolongation).

- dexamethasone
  dexamethasone will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. No dosage adjustment for ondansetron is recommended for patients on these drugs.

- dichlorphenamide
  dichlorphenamide and ondansetron both decrease serum potassium. Use Caution/Monitor.

- doxepin
  doxepin and ondansetron both increase QTc interval. Use Caution/Monitor.

- dronedarone
  dronedarone will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- efavirenz
  efavirenz will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- elagolix
  elagolix will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

  elagolix decreases levels of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Elagolix is a weak-to-moderate CYP3A4 inducer. Monitor CYP3A substrates if coadministered. Consider increasing CYP3A substrate dose if needed.

- eliglustat
  eliglustat increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Monitor therapeutic drug concentrations, as indicated, or consider reducing the dosage of the P-gp substrate and titrate to clinical effect.

- elvitegravir/cobicistat/emtricitabine/tenofovir DF
  elvitegravir/cobicistat/emtricitabine/tenofovir DF increases levels of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Cobicistat is a CYP3A4 inhibitor; contraindicated with CYP3A4 substrates

for which elevated plasma concentrations are associated with serious and/or life-threatening events.

- encorafenib
  encorafenib, ondansetron. affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Encorafenib both inhibits and induces CYP3A4 at clinically relevant plasma concentrations. Coadministration of encorafenib with sensitive CYP3A4 substrates may result in increased toxicity or decreased efficacy of these agents.

- fedratinib
  fedratinib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Adjust dose of drugs that are CYP3A4 substrates as necessary.

- fenfluramine
  ondansetron decreases effects of fenfluramine by pharmacodynamic antagonism. Use Caution/Monitor. Potent serotonin receptor antagonists may decrease fenfluramine efficacy. If coadministered, monitor appropriately.

- flibanserin
  flibanserin will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- formoterol
  ondansetron and formoterol both increase QTc interval. Use Caution/Monitor. Combination may increase risk of QT prolongation; monitor electrolytes and ECG changes

- fosamprenavir
  fosamprenavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- fostemsavir
  ondansetron and fostemsavir both increase QTc interval. Use Caution/Monitor. QTc prolongation reported with higher than recommended doses of fostemsavir.

- gadobenate
  gadobenate and ondansetron both increase QTc interval. Use Caution/Monitor.

- gemtuzumab
  ondansetron and gemtuzumab both increase QTc interval. Use Caution/Monitor.

- gepirone
  gepirone and ondansetron both increase QTc interval. Modify Therapy/Monitor Closely.

- glecaprevir/pibrentasvir
  glecaprevir/pibrentasvir will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- goserelin
  goserelin increases toxicity of ondansetron by QTc interval. Use Caution/Monitor. Increases risk of torsades de pointes.

- histrelin
  histrelin increases toxicity of ondansetron by QTc interval. Use Caution/Monitor. Increases risk of torsades de pointes.

- hydroxyzine
  hydroxyzine increases toxicity of ondansetron by QTc interval. Use Caution/Monitor. Increases risk of torsades de pointes.

- isoniazid
  isoniazid will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- istradefylline
  istradefylline will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Istradefylline 40 mg/day increased peak levels and AUC of CYP3A4 substrates in clinical trials. This effect was not observed with istradefylline 20 mg/day. Consider dose reduction of sensitive CYP3A4 substrates.

  istradefylline will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Istradefylline 40 mg/day increased peak levels and AUC of P-gp substrates in clinical trials. Consider dose reduction of sensitive P-gp substrates.

- itraconazole
  itraconazole will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. P-gp inhibitors may decrease clearance of ondansetron. No dosage adjustment for ondansetron is recommended

- ivacaftor
  ivacaftor increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Ivacaftor and its M1 metabolite has the potential to inhibit P-gp; may significantly increase systemic exposure to sensitive P-gp substrates with a narrow therapeutic index.

- ketoconazole
  ketoconazole will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- lenvatinib
  ondansetron and lenvatinib both increase QTc interval. Use Caution/Monitor. Lenvatinib prescribing information recommends monitoring ECG closely when coadministered with QT prolonging drugs.

- letermovir

letermovir increases levels of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- leuprolide
  leuprolide increases toxicity of ondansetron by QTc interval. Use Caution/Monitor. Increases risk of torsades de pointes.

- levalbuterol
  ondansetron and levalbuterol both increase QTc interval. Use Caution/Monitor. Combination may increase risk of QT prolongation; monitor electrolytes and ECG changes

- levoketoconazole
  levoketoconazole will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- lonafarnib
  lonafarnib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Lonafarnib is a weak P-gp inhibitor. Monitor for adverse reactions if coadministered with P-gp substrates where minimal concentration changes may lead to serious or life-threatening toxicities. Reduce P-gp substrate dose if needed.

- lopinavir
  lopinavir and ondansetron both increase QTc interval. Use Caution/Monitor. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias.

- lorlatinib
  lorlatinib will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- mavorixafor
  mavorixafor and ondansetron both increase QTc interval. Modify Therapy/Monitor Closely. Mavorixafor causes concentration-dependent QTc prolongation. Monitor QTc during treatment in patients with risk factors for QTc prolongation (eg, coadministered medications that increase mavorixafor exposure or other drugs with a high risk to prolong the QTc interval). Mavorixafor dose reduction or discontinuation may be required.

  mavorixafor will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Caution if mavorixafor (a P-gp inhibitor) is coadministered with a sensitive P-gp substrate where minimal substrate concentration changes may lead to serious adverse effects.

- metformin
  ondansetron increases levels of metformin by Other (see comment). Use Caution/Monitor. Comment: Ondansetron inhibits organic cation transporter-2 (OCT2) and multidrug and toxin extrusion (MATE) transporters, increasing

metformin exposure. Approach long-term concomitant use with caution; monitor for metformin-related toxicities and consider dose reduction.

- metronidazole
  ondansetron and metronidazole both increase QTc interval. Use Caution/Monitor. Caution if metronidazole is coadministered with ondansetron IV or a higher ondansetron IV dose (do not exceed 16 mg IV).

- mifepristone
  mifepristone, ondansetron. QTc interval. Modify Therapy/Monitor Closely. Use alternatives if available.

- mitotane
  mitotane decreases levels of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Mitotane is a strong inducer of cytochrome P-4503A4; monitor when coadministered with CYP3A4 substrates for possible dosage adjustments.

- nefazodone
  nefazodone will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- nelfinavir
  nelfinavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. CYP-450 inhibitors may decrease clearance of ondansetron. However, no dosage adjustment for ondansetron is recommended

- osilodrostat
  osilodrostat and ondansetron both increase QTc interval. Use Caution/Monitor.

- osimertinib
  osimertinib and ondansetron both increase QTc interval. Use Caution/Monitor. Conduct periodic monitoring with ECGs and electrolytes in patients taking drugs known to prolong the QTc interval.

- oxaliplatin
  oxaliplatin will increase the level or effect of ondansetron by Other (see comment). Use Caution/Monitor. Monitor for ECG changes if therapy is initiated in patients with drugs known to prolong QT interval.

- ozanimod
  ozanimod and ondansetron both increase QTc interval. Modify Therapy/Monitor Closely. The potential additive effects on heart rate, treatment with ozanimod should generally not be initiated in patients who are concurrently treated with QT prolonging drugs with known arrhythmogenic properties.

- panobinostat
  ondansetron and panobinostat both increase QTc interval. Modify Therapy/Monitor Closely. Panobinostat is known to significantly prolong QT interval. Panobinostat prescribing information states use with drugs known to prolong QTc is not

recommended; however, antiemetic drugs known to prolong QTc (eg, dolasetron, granisetron, ondansetron) can be used with frequent ECG monitoring.

- pasireotide
  ondansetron and pasireotide both increase QTc interval. Modify Therapy/Monitor Closely.

- pentobarbital
  pentobarbital will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. No dosage adjustment for ondansetron is recommended for patients on these drugs.

- phenobarbital
  phenobarbital will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. No dosage adjustment for ondansetron is recommended for patients on these drugs.

- phenytoin
  phenytoin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. No dosage adjustment for ondansetron is recommended for patients on these drugs.

- ponatinib
  ponatinib increases levels of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- quizartinib
  quizartinib, ondansetron. Either increases effects of the other by QTc interval. Modify Therapy/Monitor Closely. Monitor patients more frequently with ECG if coadministered with QT prolonging drugs.

- ribociclib
  ribociclib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- rifabutin
  rifabutin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- rifampin
  rifampin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- rilzabrutinib
  rilzabrutinib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Follow substrate recommendations for use with moderate CYP3A inhibitors

- ritonavir
  ritonavir and ondansetron both increase QTc interval. Use Caution/Monitor. Avoid with congenital long QT syndrome; ECG monitoring recommended with concomitant medications that prolong QT interval, electrolyte abnormalities, CHF, or bradyarrhythmias. May increase ondansetron levels.

ritonavir will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- rucaparib
  rucaparib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Adjust dosage of CYP3A4 substrates, if clinically indicated.

- sarecycline
  sarecycline will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Monitor for toxicities of P-gp substrates that may require dosage reduction when coadministered with P-gp inhibitors.

- selpercatinib
  selpercatinib increases toxicity of ondansetron by QTc interval. Use Caution/Monitor.

- sorafenib
  sorafenib and ondansetron both increase QTc interval. Use Caution/Monitor.

- St John's Wort
  St John's Wort will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor.

- stiripentol
  stiripentol, ondansetron. affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Stiripentol is a CYP3A4 inhibitor and inducer. Monitor CYP3A4 substrates coadministered with stiripentol for increased or decreased effects. CYP3A4 substrates may require dosage adjustment.

  stiripentol will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Consider reducing the dose of P-glycoprotein (P-gp) substrates, if adverse reactions are experienced when administered concomitantly with stiripentol.

- tecovirimat
  tecovirimat will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Tecovirimat is a weak CYP3A4 inducer. Monitor sensitive CYP3A4 substrates for effectiveness if coadministered.

- tramadol
  ondansetron decreases effects of tramadol by Other (see comment). Use Caution/Monitor. Comment: Ondansetron may decrease therapeutic effects of tramadol. Ondansetron may increase serotonergic effects of tramadol. This could result in serotonin syndrome. Tramadol dose requirement was significantly greater postoperatively among those who had received ondansetron than among those receiving a control.

- triclabendazole

triclabendazole and ondansetron both increase QTc interval. Use Caution/Monitor.

- triptorelin
  triptorelin increases toxicity of ondansetron by QTc interval. Use Caution/Monitor. Increases risk of torsades de pointes.

- tucatinib
  tucatinib will increase the level or effect of ondansetron by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Consider reducing the dosage of P-gp substrates, where minimal concentration changes may lead to serious or life-threatening toxicities.

- valbenazine
  valbenazine and ondansetron both increase QTc interval. Use Caution/Monitor.

- voclosporin
  voclosporin, ondansetron. Either increases effects of the other by QTc interval. Use Caution/Monitor.

Minor (28)

- acetazolamide
  acetazolamide will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- anastrozole
  anastrozole will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- armodafinil
  armodafinil will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

  armodafinil will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- bosentan
  bosentan will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- cigarette smoking
  cigarette smoking will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- conivaptan
  conivaptan will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- cyclophosphamide
  cyclophosphamide will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- danazol
  danazol will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- diltiazem
  diltiazem will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown. No dosage adjustment for ondansetron is recommended.

- drospirenone
  drospirenone will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- eslicarbazepine acetate
  eslicarbazepine acetate will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown. No dosage adjustment for ondansetron is recommended for patients on these drugs.

- etravirine
  etravirine will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- fosphenytoin
  fosphenytoin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- grapefruit
  grapefruit will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- griseofulvin
  griseofulvin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- larotrectinib
  larotrectinib will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- mexiletine
  mexiletine will increase the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- nafcillin
  nafcillin will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- nevirapine
  nevirapine will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- peginterferon alfa 2a
  peginterferon alfa 2a will increase the level or effect of ondansetron by affecting

hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- pentobarbital
  pentobarbital will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- phenobarbital
  phenobarbital will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- primidone
  primidone will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

  primidone will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- rifampin
  rifampin will decrease the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

- rifapentine
  rifapentine will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- topiramate
  topiramate will decrease the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- verapamil
  verapamil will increase the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown.

  verapamil will increase the level or effect of ondansetron by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Minor/Significance Unknown.

- zileuton
  zileuton will increase the level or effect of ondansetron by affecting hepatic enzyme CYP1A2 metabolism. Minor/Significance Unknown. No dosage adjustment is needed for ondansetron.

Adverse Effects

>10%

Headache (9-27%)

Malaise/fatigue (9-13%)

Constipation (6-11%)

1-10%

Hypoxia (9%)

Drowsiness (8%)

Diarrhea (2-7%)

Dizziness (7%)

Fever (2-8%)

Gynecologic disorder (7%)

Anxiety (6%)

Urinary retention (5%)

Pruritus (2-5%)

Injection-site pain (4%)

Paresthesia (2%)

Cold sensation (2%)

Elevated liver function test results (1-5%)

Postmarketing Reports

Cardiovascular

- Arrhythmias (including ventricular and supraventricular tachycardia, premature ventricular contractions, and atrial fibrillation), bradycardia, electrocardiographic alterations (including second-degree heart block, QT/QTc interval prolongation, and ST segment depression), palpitations, and syncope
- Rarely and predominantly with IV administration
    - Transient ECG changes, including QT/QTc interval prolongation reported
    - Myocardial ischemia

General

- Flushing: Rare cases of hypersensitivity reactions, sometimes severe (eg, anaphylactic reactions, angioedema, bronchospasm, cardiopulmonary arrest, hypotension, laryngeal edema, laryngospasm, shock, shortness of breath, stridor) reported
- A positive lymphocyte transformation test to ondansetron has been reported, which suggests immunologic sensitivity

Hepatobiliary

- Liver enzyme abnormalities reported
- Liver failure and death reported in patients with cancer receiving concurrent medications, including potentially hepatotoxic cytotoxic chemotherapy and

antibiotics

Local reactions

- Pain, redness, and burning at site of injection

Lower respiratory

- Hiccups

Neurological

- Oculogyric crisis, appearing alone, as well as with other dystonic reactions
  Transient dizziness during or shortly after IV infusion

Skin

- Urticaria, Stevens-Johnson syndrome, and toxic epidermal necrolysis

Eye disorders

- Cases of transient blindness, predominantly during IV administration, reported
- These cases of transient blindness were reported to resolve within a few minutes up to 48 hr
- Transient blurred vision reported, in some cases associated with abnormalities of accommodation

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Warnings

Contraindications

Hypersensitivity

Coadministration with apomorphine; combination reported to cause profound hypotension and loss of consciousness

Cautions

Hypersensitivity reactions including anaphylaxis and bronchospasm may occur: discontinue therapy if suspected; monitor and treat promptly per standard of care until signs and symptoms resolve

Reduce dose with severe hepatic impairment

Use according to schedule, not PRN

Do not use instead of nasogastric suction

Ondansetron may mask progressive ileus or gastric distention in patients who are undergoing abdominal surgery or experiencing chemotherapy-induced nausea and vomiting; monitor for decreased bowel activity, particularly in patients with risk factors for gastrointestinal obstruction

Ondansetron is not a drug that stimulates gastric or intestinal peristalsis; should not be used instead of nasogastric suction

Serotonin syndrome reported with 5-HT3 receptor antagonists alone but particularly with concomitant use of serotonergic drugs including SSRIs, SNRIs, MAO inhibitors, lithium, tramadol, methylene blue IV, and mirtazapine; if concomitant use with other serotonergic drugs is clinically warranted, patients should be made aware of potential increased risk for serotonin syndrome

Cross-sensitivity among selective serotonin antagonists may occur

Zofran ODT contains phenylalanine (caution for phenylketonurics)

Dose-dependent QT prolongation; avoid in patients with congenital long QT syndrome; ECG monitoring recommended in patients who have electrolyte abnormalities, CHF, or bradyarrhythmias or who are also receiving other medications that cause QT prolongation

Myocardial ischemia

- Myocardial ischemia reported in patients treated with ondansetron; in some cases, predominantly during intravenous administration, the symptoms appeared immediately after administration but resolved with prompt treatment
- Coronary artery spasm appears to be the most common underlying cause; monitor or advise patients for signs or symptoms of myocardial ischemia after oral administration of ondansetron

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Pregnancy & Lactation

Pregnancy

Published epidemiological studies on association between ondansetron use and major birth defects have reported inconsistent findings and have important methodological limitations that preclude conclusions about safety use in pregnancy

Available postmarketing data have not identified a drug-associated risk of miscarriage or adverse maternal outcomes

Animal studies

- Reproductive studies in rats and rabbits did not show evidence of harm to fetus when ondansetron was administered IV during organogenesis at ~3.6 and 2.9x times maximum recommended human oral dose of 0.15 mg/kg, based on body surface area, respectively

Lactation

Unlikely to result in clinically relevant exposures in breastfed infants when administered IV at doses up to 4 mg/day to women who are breastfeeding

Available data from a lactation study involving pharmacokinetic samples from 80 lactating women and 20 infants indicate that ondansetron is present at low levels in human milk and in plasma of breastfed infants

In this study, no adverse effects attributed to ondansetron were reported in infants exposed to ondansetron through breast milk

There are no data regarding effects on milk production

Consider developmental and health benefits of breastfeeding along with mother's clinical need for drug and any potential adverse effects on breastfed infants from therapy or from underlying maternal condition

Pregnancy Categories

A: Generally acceptable. Controlled studies in pregnant women show no evidence of fetal risk.

B: May be acceptable. Either animal studies show no risk but human studies not available or animal studies showed minor risks and human studies done and showed no risk.

C: Use with caution if benefits outweigh risks. Animal studies show risk and human studies not available or neither animal nor human studies done.

D: Use in LIFE-THREATENING emergencies when no safer drug available. Positive evidence of human fetal risk.

X: Do not use in pregnancy. Risks involved outweigh potential benefits. Safer alternatives exist.

NA: Information not available.

Pharmacology

Mechanism of Action

Mechanism not fully characterized; selective 5-HT3 receptor antagonist; binds to 5-HT3 receptors both in periphery and in CNS, with primary effects in GI tract

Has no effect on dopamine receptors and therefore does not cause extrapyramidal symptoms

Absorption

Bioavailability: 56-71% (PO); food increases extent of absorption (17%)

Onset: 30 min

Peak plasma time: IV, end of infusion; IM, 30 min; PO, 2 hr (tablet) or 1 hr (soluble film)

Distribution

Protein bound: 70-76%

Vd: Children, 1.7-3.7 L/kg; adults, 2.2-2.5 L/kg

Metabolism

Extensive hepatic metabolism, with hydroxylation followed by glucuronide (indole ring) or sulfate conjugation; metabolized by CYP2D6 and partly by CYP1A2 and CYP3A4

Metabolites: Glucuronide conjugate, sulfate conjugate (inactive)

Elimination

Half-life: 2-7 hr (children <15 years); 3-7 hr (adults); patients with mild to moderate hepatic impairment, 12 hr; patients with severe hepatic impairment (Child-Pugh class C), 20 hr

Renal Clearance: 0.26-0.38 L/hr/kg

Total body clearance: 600-700 mL/min

Excretion: Primarily urine (30-70%); feces (25%)

Administration

IV Incompatibilities

Syringe: Droperidol

Y-site: Acyclovir, allopurinol, aminophylline, amphotericin B, amphotericin B cholesteryl sulfate, ampicillin, ampicillin/sulbactam, amsacrine, cefepime, cefoperazone, 5-fluorouracil (5-FU; at 1 mg/mL ondansetron and 16 mg/mL 5-FU; may be compatible at 0.8 mg/mL 5-FU and up to 160 mcg/mL ondansetron), furosemide, ganciclovir, lorazepam, meropenem (at 50 mg/mL meropenem and 1 mg/mL ondansetron; may be compatible at 1 mg/mL each), methylprednisolone, piperacillin, sargramostim, sodium bicarbonate

IV Compatibilities

Solution: Compatible with most common solvents

Additive (partial list): Cisplatin, cyclophosphamide, cytarabine, decarbazine, dacarbazine with doxorubicin(?), doxorubicin, etoposide, hydromorphone, meropenem (incompatible at 20 g/L meropenem), methotrexate, morphine sulfate

Syringe: Alfentanil, atropine, dexamethasone (incompatible at 0.67 mg/mL dexamethasone and 1.07 mg/mL ondansetron), fentanyl, glycopyrrolate, meperidine, metoclopramide, midazolam, morphine sulfate, naloxone, neostigmine, propofol

Y-site (partial list): Alatrofloxacin, aldesleukin, azithromycin, bleomycin, carboplatin, cisplatin, cladribine, clindamycin, cyclophosphamide, cytarabine, dactinomycin, dopamine, heparin, hydromorphone, magnesium sulfate, meperidine, morphine sulfate, paclitaxel, potassium chloride, topotecan, vancomycin, vinblastine, vincristine, zidovudine

IV Preparation

No dilution necessary for premixed injection

Postoperative nausea and vomiting: No dilution necessary for 2 mg/mL vials

Chemotherapy-induced nausea and vomiting: Dilute IV injection (2 mg/mL vials, not premixed injection) in 50 mL D5W or NS

IV Administration

Infuse over 15 minutes after further dilution with 50 mL NS/D5W

Inject undiluted over at least 30 seconds, preferably over 2-5 minutes

IM Administration

No dilution necessary for premixed injection

Storage

Store at room temperature or refrigerate

Protect from light, excessive heat, and freezing

Formulary

Adding plans allows you to compare formulary status to other drugs in the same class.

Formulary information is available for health plans only in the United States.